

Report to **IMEDICAL,**
1 Union st
, Pyrmont, NSW, 2009

Patient **SCHLEGER, SHANE**
IMEDICAL C/O IMEDICAL 1 UNION ST PYRMONT NSW 2009

Phone
D.O.B 01/01/1985 Age 39 years Sex M

Ref. by/copy to IMEDICAL, Collect date 19/11/2024 Lab ref 24-10636734
Collect time 10:56 AM Your ref
Reported 20/11/2024 01:52 PM

Tests requested GH, SCP, SOM, HOR, PRL, AND, COR, LIP, MBA, FE, TFT, GLU, FBE

Clinical notes

SERUM GROWTH HORMONE

Growth Hormone < 1.0 mIU/L (< 3.1)

In children, Growth Hormone should be > 20 mIU/L following any appropriate stimulus (e.g. sleep, exercise, hypoglycaemia, arginine). Random values are usually low. Growth Hormone is not recommended to diagnose Growth Hormone excess or deficiency. IGF-1 is the preferred marker, especially for the investigation of Growth Hormone excess.

**SURGERY
USE**

- Normal
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- Patient Notified
- Make Appoint.
- Further Tests
- Notes Required
- Speak with Dr.
- On Correct Treatment

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SERUM HIGH SENSITIVITY C-REACTIVE PROTEIN (CRP)

Request Number 10636734
Date Collected 19 Nov 24
Time Collected 10:56
hsCRP (< 4.91) mg/L 0.79

The CDC / AHA recommend the following hsCRP cut-off points (tertiles) for cardiovascular disease risk assessment:

hsCRP level (mg/L)	Relative Risk
<1.0	Low
1.0 - 3.0	Average
>3.0	High

The average of two CRP tests, ideally taken two weeks apart, produces a more stable estimate of this marker.

A CRP greater than 10mg/L should prompt a search for a source of infection or inflammation.

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INSULIN-LIKE GROWTH FACTOR-1 (IGF-1)

Specimen Type: Serum

IGF-1 (Somatomedin C) 18 nmol/L (13-36)

Insulin-like growth factor 1 (IGF1) and insulin-like growth factor-binding protein 3 (IGFBP3) measurements can be used to assess growth hormone (GH) excess or deficiency. For all applications, IGF1 measurement has generally been shown to have superior diagnostic accuracy. In particular, for the diagnosis and follow-up of acromegaly and gigantism, IGFBP3 measurement adds little extra diagnostic information.

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SERUM HORMONE PROFILE

Specimen Type: Serum							
Request	Date	FSH	LH	PROG	E2(ATEL)	E2(BECK)	LH/FSH
Number	Collected	IU/L	IU/L	nmol/L	pmol/L	pmol/L	Ratio
10636734	19 Nov 24	14	6.1	2	100		
Reference Ranges		FSH	LH	PROG	E2(ATEL)		
Males < 60 yrs		<7	<7	0.9-3.9	<150		
> 60 yrs		<14	<11	0.9-3.9	<150		

PLEASE NOTE:
'E2 (ATEL)' - Oestradiol by Siemens Atellica assay
'E2 (BECK)' - Oestradiol by Beckman Access assay

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PROLACTIN			
Specimen Type: Serum			
Request Number	Date Collected	Ref Range	Prolactin mIU/L (40-450)
10636734	19/11/24		267

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☐ Normal

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☐ Further Tests

☐ Notes
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SERUM ANDROGENS

Total Testosterone (Siemens)	16.5	nmol/L	(8.3-29)
Sex Hormone Binding Globulin	40	nmol/L	(11-71)
DHEAS	7.8	umol/L	(3.0-10)
Calculated Free Testosterone	295.3	pmol/L	(255.0-725.0)

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SERUM CORTISOL

Time	10:36	
Cortisol	466	nmol/L

AM Reference Interval 120-620 nmol/L

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LIPID STUDIES

Specimen Type: Serum

Reference intervals are included for reference only, and interpretation / treatment goals should be guided by patient-specific cardiovascular risk assessment (see Australian Cardiovascular Risk Charts. Alternatively, the web-site www.cvdcheck.org.au can be accessed in order to complete a risk assessment for individual patients.)

Haemolysis Nil
Icterus Nil
Lipaemia Nil

Fasting status	Fasting		
Total Cholesterol	4.3	mmol/L	(3.6-5.2)
Triglycerides	1.1	mmol/L	(0.5-1.7)
HDL Cholesterol	1.1	mmol/L	(1.0-2.0)
LDL Cholesterol	2.7	mmol/L	(1.5-3.4)
Non-HDL Cholesterol	3.2	mmol/L	(< 3.4)
Cholesterol/HDL-C Ratio	3.9		(< 5.0)

NVDPA TARGET LIPID RANGES (MMOL/L) FOR PATIENTS AT HIGH / MODERATE RISK OF CARDIOVASCULAR DISEASE:

TOTAL CHOLESTEROL	<4.0
TRIGS (FASTING)	<2.0
HDL-C	>= 1.0
LDL-C	<2.0
NON HDL-C	<2.5

LDL-C exceeds target for higher risk patients and may be excessive in some individuals.

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SERUM CHEMISTRY

Specimen Type: Serum

Haemolysis	Nil		
Icterus	Nil		
Lipaemia	Nil		
Sodium	141	mmol/L	(135-145)
Potassium	4.0	mmol/L	(3.6-5.4)
Chloride	101	mmol/L	(95-110)
Bicarbonate	26	mmol/L	(22-32)
Anion Gap	18	mmol/L	(10-20)
Urea	5.7	mmol/L	(2.5-7.5)
Creatinine	100	umol/L	(60-110)
eGFR	79		mL/min/1.73sqM
Urate	0.43	mmol/L	(0.20-0.42)
Bilirubin	17	umol/L	(< 20)
AST	22	U/L	(< 40)
ALT	27	U/L	(< 40)
GGT	20	U/L	(< 40)
Alkaline Phosphatase	62	U/L	(35-110)
Protein	74	g/L	(60-82)
Albumin	46	g/L	(38-50)
Globulin	28	g/L	(20-38)
Calcium	2.50	mmol/L	(2.10-2.60)
Corrected Calcium	2.44	mmol/L	(2.10-2.60)
Phosphate	1.03	mmol/L	(0.75-1.50)
Magnesium	0.86	mmol/L	(0.70-1.10)

eGFR values between 60 and 89 mL/min/1.73m² should be interpreted with caution. These results are only consistent with CKD in the presence of other evidence such as microalbuminuria, proteinuria or haematuria.
Ref:Lamb EJ etal in Ann Clin Biochem 2005; 42:321-345.

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IRON STUDIES

Specimen Type: Serum			
Serum Iron	26	umol/L	(10-30)
Transferrin	28	umol/L	(27-46)
Transferrin Saturation	47	%	(13-45)
Serum Ferritin	194	ug/L	(30-300)

Raised transferrin saturation may be the earliest sign of iron overload.
Suggest repeat fasting iron studies to confirm the finding.

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THYROID PROFILE

Specimen Type: Serum			
TSH	1.4	mIU/L	(0.5-4.0)
FT4	17	pmol/L	(10-20)
FT3	5.9	pmol/L	(3.5-6.5)

Result(s) consistent with euthyroidism.

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SERUM/PLASMA GLUCOSE
Fasting status Fasting
Serum 5.1 mmol/L (3.4-5.4)

Normal glucose concentration.

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HAEMATOLOGY

Date Collected 19 Nov 24
Time Collected 10:56
Specimen Type: EDTA

Hb	157	g/L	(130-180)	WBC	6.0	x10 ⁹	/L (4.0-11.0)
RCC	5.2	x10 ¹²	/L (4.5-6.5)	Neut	2.7	x10 ⁹	/L (2.0-7.5)
Hct	0.45		(0.40-0.54)	Lymp	2.6	x10 ⁹	/L (1.0-4.0)
MCV	87	fL	(79-99)	Mono	0.4	x10 ⁹	/L (0.2-1.0)
MCH	30	pg	(27-34)	Eos	0.3	x10 ⁹	/L (< 0.7)
MCHC	351	g/L	(320-360)	Baso	0.1	x10 ⁹	/L (< 0.2)
RDW	12.1	%	(10.0-17.0)				
Plat	284	x10 ⁹	/L (150-400)				

HAEMATOLOGY: FBC parameters are within reference range.

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